

aml-mds-related-rr-tp53-aberrant-hla-pending-x7q2-rerun

Standard of care options — approved and guideline-endorsed strategies

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Standard of care options

The treatment strategies that are standard for this patient's situation, meaning a regulator approved them for a population that includes this patient or a major academic or clinical-society guideline carries them. This report runs alongside the targetable-feature ranking rather than in place of it, and it does not narrow what that ranking surfaces.

Assessed: 12 standard options. **Consider now:** 6. **Behind an open gate:** 0. **Already received:** 2.

Twelve standard options were screened for this relapsed/refractory AML-MR, and six are worth weighing now. The curative-intent backbone the guidelines carry for AML that has relapsed after an allograft is a second allogeneic transplant, with donor lymphocyte infusion from the same HLA-identical sibling as the other cellular route at that fork. NCCN and the ELN 2022 panel keep both in play for fit patients, and NCCN makes clinical trial enrollment the preferred strategy in this setting. Gemtuzumab ozogamicin earns a row on its own footing: it is FDA-approved for relapsed/refractory CD33-positive AML and these blasts are CD33-positive, so it is standard here by regulatory maturity, though its own tier-1a evidence shows no survival benefit in the adverse-risk cytogenetics this patient carries. High-dose cytarabine salvage and radiotherapy to the thigh myeloid sarcoma fill out the near-term list as a cytoreductive bridge and local control.

Two standard options are already spent: the first sibling allograft, which held remission for about six years, and the FLAG-IDA plus venetoclax reinduction the disease progressed through.

No standard option sits behind an open biomarker gate, but the transplant backbone is conditioned on a fitness and organ-function workup that has not been done. The two approved menin inhibitors, revumenib and ziftomenib, are set aside on target: this disease carries KMT2A amplification, not the KMT2A translocation the revumenib label requires, and is NPM1 wild-type.

This page is additive. It lists the standard options and does not narrow, reorder, or filter the targeted options ranked elsewhere on the case page.

Options to consider now

Standard options this patient is eligible for, with an endorsement that covers their situation, and no unresolved gate in front of them.

Clinical trial enrollment

Priority: high **Modality:** Other **Line:** Salvage **Intent:** curative intent

What makes it standard.

- NCCN Preferred approach for relapsed/refractory AML (covers this patient · NCCN AML v3.2026)
 - covers: Relapsed/refractory AML: clinical trial enrollment is preferred whenever available, given the limited proven options in this setting.
- other_society ELN 2022: Expert-panel recommendation (covers this patient · ELN 2022)
 - covers: Relapsed/refractory AML with active disease or early relapse: patients should be considered for clinical trials.

Fit to this patient. Likely eligible Performance status is ECOG 1, curative intent and a preference for trials are on file, and travel is acceptable; eligibility for any specific protocol still depends on that protocol's criteria and on the pending HLA and TP53 workup.

NCCN and the ELN 2022 panel both make clinical trial enrollment the preferred strategy for relapsed/refractory AML, because the proven off-trial options in this setting are limited. For this patient it is also the delivery route for the case's motivating HLA-restricted cellular options. Specific eligibility hinges on the pending HLA typing and TP53 resolution, which is why the diagnostic workup is the ranked first step.

Toxicities that would change the decision. Protocol-specific; varies by the individual study

Alongside the targeted options. Can run alongside the targeted options. Every ranked targeted option in this case, from the HLA and HA-1 workup through the HA-1 TCR-T and the CD123 agents, is delivered through a trial or expanded-access route, so this standard recommendation is the mechanism by which those options reach the patient rather than a competitor to them.

Last verified. 2026-07-24

References. guideline:NCCN AML v3.2026; pmid:35797463

Second allogeneic hematopoietic cell transplant (HCT2)

Priority: high **Modality:** Transplant **Line:** Salvage **Intent:** curative intent

What makes it standard.

- NCCN Category 2A (transplant / clinical-trial route) (covers this patient · NCCN AML v3.2026)
 - covers: AML relapse after prior allogeneic HCT: a second allo-HCT or donor lymphocyte infusion is an option for fit patients, best pursued after remission or maximal cytoreduction; active disease and early relapse favour a trial.
- other_society ELN 2022: Expert-panel recommendation (covers this patient · ELN 2022)
 - covers: Post-transplant AML relapse: DLI and second allo-HCT are treatment options for eligible patients; shorter time to relapse and disease not in remission predict worse outcomes.

Fit to this patient. Conditionally eligible The late relapse roughly six years out is a favourable selection factor, but the profile records no cardiac, renal, or hepatic function, and the marrow is around 85% blasts with extramedullary disease, so remission or cytoreduction is needed before conditioning.

- blocker: No cardiac, renal, or hepatic function on file; transplant fitness is undetermined and flagged as an open item
- blocker: Active high-burden disease (~85% marrow blasts) not in remission; cytoreduction is needed before conditioning
- blocker: Donor availability and consent for a second graft from the sibling not yet confirmed in the record

A second allogeneic transplant is the standard curative-intent route for AML that has relapsed after a first allo-HCT, and both NCCN and the ELN 2022 panel keep it in play for fit patients. It is the platform the case's curative plan is built around, including the ranked HA-1 TCR-T consolidation. The load-bearing caveats are that transplant fitness is undocumented and the marrow is not in remission, so cytoreduction and an organ-function workup have to come first.

Key evidence.

- How I treat AML relapse after allo-HCT (Blood 2025) · expert review of second-HCT and DLI outcomes · Second allo-HCT and DLI both offer curative potential in selected fit patients; reported long-term survival after post-transplant relapse remains on the order of 20-30% and is worse with active disease or early relapse.

Toxicities that would change the decision. Non-relapse mortality from a second conditioning and from graft-versus-host disease; Regimen-related organ toxicity, infection, and second-transplant mortality that is not well characterized after a prior allo-HCT

Alongside the targeted options. Comes before the targeted options. The HCT2 platform is the base on which the ranked HA-1 TCR-T consolidation would be delivered, and the ranked lomab-B option is a radioimmunotherapy way to condition for this same transplant. The standard transplant comes first; the targeted cellular consolidation follows on it.

Last verified. 2026-07-24

References. guideline:NCCN AML v3.2026; pmid:35797463; pmid:39719042

Donor lymphocyte infusion (DLI) from the HLA-identical sibling donor

Priority: medium **Modality:** Cellular therapy **Line:** Salvage **Intent:** disease-control intent

What makes it standard.

- NCCN Category 2A (covers this patient · NCCN AML v3.2026)
 - covers: AML relapse after allogeneic HCT: donor lymphocyte infusion is an option in fit patients, generally after cytoreduction, to re-establish a graft-versus-leukemia effect.
- other_society ELN 2022: Expert-panel recommendation (covers this patient · ELN 2022)
 - covers: Post-transplant AML relapse: DLI is a treatment option for eligible patients, with response tied to lower disease burden.

Fit to this patient. Conditionally eligible The sibling donor is available in principle, but DLI against an ~85% blast marrow rarely responds without prior cytoreduction, and the relapse pattern reads as graft-versus-leukemia escape, which tempers the expected benefit of re-stimulating the same graft.

- blocker: Active high-burden disease (~85% blasts); DLI response is poor without prior cytoreduction
- blocker: Graft-versus-host disease risk
- blocker: Relapse pattern consistent with graft-versus-leukemia escape, which predicts limited benefit from DLI given as monotherapy

Donor lymphocyte infusion from the HLA-identical sibling is a standard, guideline-carried option for AML relapse after transplant, aimed at re-establishing a graft-versus-leukemia effect. It sits at the same fork as a second transplant and shares a mechanism with the ranked HA-1 TCR-T. The caveats are the high blast burden, which blunts DLI given alone, and the graft-versus-leukemia escape pattern that produced this relapse.

Key evidence.

- How I treat AML relapse after allo-HCT (Blood 2025) · expert review of DLI outcomes · DLI alone produces a subsequent remission in roughly a quarter to a third of post-transplant AML relapses, with 2-year survival near

25%; results are better at low disease burden.

Toxicities that would change the decision. Acute and chronic graft-versus-host disease; Marrow aplasia if given without a supporting graft

Alongside the targeted options. Competes for the same line of therapy. DLI and a second allo-HCT are the two standard cellular routes at this fork, and DLI shares the graft-versus-leukemia mechanism with the ranked HA-1 TCR-T but without the HLA-restricted antigen targeting. Choosing DLI now uses the same donor and the same window the transplant-plus-TCR-T plan depends on.

Last verified. 2026-07-24

References. guideline:NCCN AML v3.2026; pmid:35797463; pmid:39719042

Gemtuzumab ozogamicin (CD33 antibody-drug conjugate)

Priority: medium **Modality:** Targeted therapy **Line:** Salvage **Intent:** disease-control intent

What makes it standard.

- FDA On-label approval (single agent) (covers this patient · FDA approval 2017-09)
 - covers: Relapsed or refractory CD33-positive acute myeloid leukemia in adults and in pediatric patients 2 years and older; single-agent dosing 3 mg/m² on days 1, 4, and 7.
- NCCN Category 2A (covers this patient · NCCN AML v3.2026)
 - covers: Relapsed/refractory CD33-positive AML: single-agent gemtuzumab ozogamicin.

Fit to this patient. Conditionally eligible The blasts are CD33-positive by flow, so the label is met on target, but baseline bilirubin and transaminases are not on file and the veno-occlusive-disease risk compounds a planned second transplant, so hepatic clearance is a prerequisite.

- blocker: Boxed warning for hepatotoxicity and veno-occlusive disease; no baseline liver function on file, and VOD risk stacks with a planned second transplant
- blocker: Tier-1a individual-patient-data meta-analysis shows no survival benefit in adverse-risk cytogenetics, the patient's exact complex, TP53-aberrant subgroup

Biomarker gate. CD33 expression CD33-positive by flow cytometry (threshold met)

Gemtuzumab ozogamicin is on-label for relapsed/refractory CD33-positive AML and NCCN-listed at category 2A, and the patient's blasts are CD33-positive, so this is a genuine standard option rather than an extrapolation. It is a dual-nature agent that the feature-targeted ranking also carries. The caveat that governs its use here is that its own tier-1a evidence shows no survival benefit in adverse-risk cytogenetics, the patient's exact subgroup, and its hepatic and VOD toxicity is the wrong risk to invite right before a second transplant.

Key evidence.

- Sievers 2001 (MyloFrance lineage) · single-arm phase 2, first-relapse CD33+ AML · n=142 · CR/CRp 30%
- Hills 2014 IPD meta-analysis · individual-patient-data meta-analysis of gemtuzumab added to induction · n=3325 · No survival benefit in adverse cytogenetics: OR 0.99 (95% CI 0.83-1.18); read across from the induction setting, not single-agent salvage

Toxicities that would change the decision. Grade 3+ hyperbilirubinemia (~23%) and transaminase elevation

(~17%); Veno-occlusive disease / sinusoidal obstruction syndrome, a class risk ahead of a second transplant

Alongside the targeted options. Competes for the same line of therapy. This is the same CD33 agent the feature-targeted ranking carries at rank 5. It appears in both tables because it is FDA-approved for R/R CD33+ AML and it also targets a feature the patient has, so the standard-of-care table logs it by regulatory maturity while the targeted ranking weighs it by mechanism. Spending the salvage slot on it is the trade-off the board flagged against the immunotherapy pathway.

Last verified. 2026-07-24

References. fda:Mylotarg; pmid:29650683; pmid:11432892; pmid:25008258; guideline:NCCN AML v3.2026

Intensive salvage chemotherapy (high-dose cytarabine-based, e.g. MEC or CLAG-M)

Priority: medium **Modality:** Systemic chemotherapy **Line:** Salvage **Intent:** disease-control intent

What makes it standard.

- NCCN Category 2A (partial population match · NCCN AML v3.2026)
 - covers: Relapsed/refractory AML in fit patients: aggressive salvage regimens incorporating high-dose cytarabine (e.g. MEC, CLAG-M) as a route toward remission or transplant.
- other_society ELN 2022: Expert-panel recommendation (partial population match · ELN 2022)
 - covers: Relapsed/refractory AML: intensive salvage chemotherapy for fit patients aiming at remission and transplant; patients with active disease or early relapse should be considered for trials.

Fit to this patient. Conditionally eligible The disease state fits the guideline, but the patient is refractory to FLAG-IDA, itself a high-dose cytarabine and fludarabine salvage, so cross-resistance limits what another intensive regimen is likely to achieve beyond cytoreduction.

- blocker: Refractory to prior FLAG-IDA (a high-dose cytarabine / fludarabine salvage), so cross-resistance limits expected response
- blocker: Cumulative marrow and organ toxicity; fitness and organ function are undocumented

High-dose cytarabine-based salvage regimens such as MEC or CLAG-M are the standard intensive option for fit relapsed/refractory AML and are the usual way to cytoreduce toward a transplant. The reason it is a bridge rather than a destination here is that the patient is already refractory to FLAG-IDA, a related high-dose cytarabine salvage, so cross-resistance caps the expected depth of response. Its value in this case is debulking the marrow ahead of the curative transplant and cellular plan.

Key evidence.

- NCCN-listed intensive salvage regimens · guideline-referenced practice, retrospective salvage series · Second-line intensive salvage CR rates are generally 20-40% and lower after prior high-dose cytarabine failure; used here mainly as a cytoreductive bridge

Toxicities that would change the decision. Prolonged cytopenias, febrile neutropenia, and infection; Mucositis and cumulative anthracycline / cardiac exposure

Alongside the targeted options. Comes before the targeted options. Intensive salvage functions here as cytoreduction to bring the ~85% blast marrow down toward a state where the ranked transplant and

cellular-immunotherapy options become deliverable. It occupies the same debulking role the ranked CD123 agents are also being weighed for.

Last verified. 2026-07-24

References. guideline:NCCN AML v3.2026; pmid:35797463

Radiotherapy to the extramedullary (right-thigh) myeloid sarcoma

Priority: low **Modality:** Radiotherapy **Line:** Any line **Intent:** symptom palliation

What makes it standard.

- NCCN Category 2A (myeloid sarcoma management) (partial population match · NCCN AML v3.2026)
 - covers: Myeloid sarcoma (extramedullary AML): local radiotherapy for symptomatic or bulky disease or where systemic therapy gives inadequate local control.

Fit to this patient. Likely eligible Radiotherapy to a defined soft-tissue site is deliverable regardless of the systemic marrow burden; it addresses only the right-thigh lesion and does not touch the dominant marrow disease.

Local radiotherapy is the standard option for a symptomatic or bulky myeloid sarcoma and would give local control at the right-thigh site. It is palliative and local by nature, so it does nothing for the ~85% blast marrow that drives the prognosis. It is worth considering now only if the thigh lesion becomes symptomatic, and it can run alongside systemic and cellular therapy.

Key evidence.

- Radiotherapy for myeloid sarcoma (retrospective series) · retrospective, single-modality local control · Durable local control at modest doses (commonly 20-30 Gy); no effect on systemic marrow disease or survival

Toxicities that would change the decision. Local skin and soft-tissue reaction and fatigue; No effect on the dominant systemic marrow disease

Alongside the targeted options. Independent of the targeted options. Radiotherapy treats the local myeloid sarcoma only and does not interact with the systemic marrow-directed targeted options; it can proceed in parallel for local control or symptom relief.

Last verified. 2026-07-24

References. guideline:NCCN AML v3.2026

Options at the next decision point

Standard for a line this patient has not reached yet, or for a disease state that has not arrived. Listed now so the sequence is visible in advance.

Best supportive care and hospice referral

Priority: low **Modality:** Supportive care **Line:** Not line-indexed **Intent:** symptom palliation

What makes it standard.

- NCCN Named management strategy (covers this patient · NCCN AML v3.2026)
 - covers: Relapsed/refractory AML: best supportive care, including transfusions, infection management, and hospice, is an option where curative therapy is not deliverable or not wanted.
- other_society ELN 2022: Expert-panel recommendation (covers this patient · ELN 2022)
 - covers: Best supportive care is appropriate for patients not proceeding to intensive or trial therapy.

Fit to this patient. Eligible Supportive care is available to any patient; it is offered here as the standard alternative if the curative pathway proves undeliverable or is declined once fitness and goals are clarified.

Best supportive care with hospice is a guideline-named management strategy for relapsed/refractory AML, and it belongs on the page as a real option rather than a footnote. Curative intent is currently on file, and fitness and goals-of-care assessment is an explicit open item, so this sits at the next decision point rather than now. It becomes the primary path if the transplant and cellular plan proves undeliverable or is declined.

Toxicities that would change the decision. None from the strategy itself; the trade-off is forgoing disease-directed therapy

Alongside the targeted options. Independent of the targeted options. Best supportive care is the alternative if the curative transplant and cellular plan cannot be delivered or is not wanted. It does not compete with any single ranked option; it is the path taken if that pathway closes.

Last verified. 2026-07-24

References. guideline:NCCN AML v3.2026; pmid:35797463

Already received

Carried so the report reads as a complete account of the standard options rather than a list that quietly omits what has been tried.

FLAG-IDA + venetoclax reinduction

Priority: low **Modality:** Systemic chemotherapy **Line:** Salvage **Intent:** curative intent

What makes it standard.

- NCCN Category 2A (covers this patient · NCCN AML v3.2026)
 - covers: Relapsed/refractory AML in fit patients: high-intensity salvage (FLAG-IDA) with or without venetoclax.

Fit to this patient. Already received

Prior exposure. Given as reinduction for relapsed/refractory AML. Best response was progressive disease; the marrow rose to ~85% blasts with extramedullary spread. This refractoriness to a high-dose cytarabine and venetoclax regimen is why re-treating with another venetoclax- or FLAG-based intensive regimen is not expected to add benefit.

FLAG-IDA with venetoclax is a standard high-intensity salvage regimen for fit relapsed/refractory AML, and it was the reinduction this patient received. It failed, with progression to an ~85% blast marrow. It is recorded here so the report reads honestly about why another venetoclax-containing or high-dose cytarabine salvage is not offered back as a fresh option.

Last verified. 2026-07-24

References. guideline:NCCN AML v3.2026

First matched-sibling allogeneic transplant (HCT1)

Priority: low **Modality:** Transplant **Line:** Not line-indexed **Intent:** curative intent

What makes it standard.

- NCCN Category 1 (curative modality for high-risk MDS) (covers this patient · NCCN MDS / AML v3.2026)
 - covers: High-risk MDS (IPSS-R high / very-high): allogeneic HCT is the only potentially curative therapy in eligible patients.
- other_society ELN 2022: Expert-panel recommendation (covers this patient · ELN 2022)
 - covers: Allogeneic HCT is curative-intent standard for eligible patients with high-risk myeloid disease.

Fit to this patient. Already received

Prior exposure. HLA-identical sibling PBSC allograft around 2020 for RCMD, IPSS-R high/very-high MDS. Best response was complete remission, sustained about 72 months. The late relapse roughly six years out shows a graft-versus-leukemia escape pattern rather than graft failure, which is part of why a second transplant and post-transplant cellular consolidation are being weighed.

The first matched-sibling allograft was the curative-intent standard for the high-risk MDS that preceded this AML, and it held a complete remission for about six years. It is logged here so the record reads as a complete account rather than one that omits the transplant the current plan builds on. Its durable graft-versus-leukemia effect and eventual escape shape the rationale for a second transplant.

Last verified. 2026-07-24

References. guideline:NCCN; pmid:35797463

Assessed and set aside

Screened against this patient and found not to apply. Listed for the audit trail.

Revumenib (menin inhibitor)

Priority: low **Modality:** Targeted therapy **Line:** Salvage **Intent:** disease-control intent

What makes it standard.

- FDA On-label approval (two indications) (different population · FDA approvals 2024-11 (KMT2A), 2025-10 (NPM1))

- covers: Relapsed or refractory acute leukemia with a KMT2A translocation (adult and pediatric ≥ 1 year), and relapsed/refractory AML with a susceptible NPM1 mutation.
- NCCN Category 2A (KMT2A-rearranged or NPM1-mutant R/R AML) (different population · NCCN AML v3.2026)
 - covers: Relapsed/refractory AML with a KMT2A rearrangement or an NPM1 mutation.

Fit to this patient. Likely ineligible The patient's karyotype shows KMT2A amplification, a copy-number gain of four copies, not the KMT2A translocation the label requires, and the disease is NPM1 wild-type. Menin inhibitor activity depends on the menin-KMT2A dependency created by KMT2A fusions or NPM1 mutations, which gene amplification does not produce.

- blocker: KMT2A amplification is a copy-number gain, not the KMT2A translocation / rearrangement the label and companion diagnostic require
- blocker: NPM1 wild-type, so the separate NPM1-mutant indication is also not met

Biomarker gate. KMT2A rearrangement or NPM1 mutation KMT2A translocation/rearrangement OR susceptible NPM1 mutation (threshold not met)

Revumenib is an approved menin inhibitor for R/R acute leukemia with a KMT2A translocation and for R/R NPM1-mutant AML, and it is worth naming precisely because this patient's KMT2A finding is easy to misread. The karyotype shows KMT2A amplification, a copy-number gain, not the KMT2A translocation the label and its companion diagnostic require, and the disease is NPM1 wild-type. Neither indication covers this patient, so revumenib is assessed and set aside.

Key evidence.

- AUGMENT-101 (KMT2A-rearranged R/R acute leukemia) · phase 2, single-arm · CR+CRh 21.2%, median duration of CR+CRh 6.4 months

Toxicities that would change the decision. Differentiation syndrome and QTc prolongation, the menin-inhibitor class effects

Alongside the targeted options. Independent of the targeted options. Named to close the loop on the KMT2A-amplification versus KMT2A-rearrangement distinction, a common point of confusion, since this genotype does not confer menin-inhibitor sensitivity. It does not interact with the ranked options.

Last verified. 2026-07-24

References. fda:Revuforj; pmid:39121437; pmid:36922593; guideline:NCCN AML v3.2026

Ziftomenib (menin inhibitor)

Priority: low **Modality:** Targeted therapy **Line:** Salvage **Intent:** disease-control intent

What makes it standard.

- FDA On-label approval (different population · FDA approval 2025-11)
 - covers: Adults with relapsed or refractory AML with a susceptible NPM1 mutation who have no satisfactory alternative treatment options.
- NCCN Category 2A (NPM1-mutant R/R AML) (different population · NCCN AML v3.2026)
 - covers: Relapsed/refractory AML with a susceptible NPM1 mutation.

Fit to this patient. Likely ineligible The disease is NPM1 wild-type, and the ziftomenib label is limited to susceptible NPM1-mutated R/R AML. The KMT2A amplification present here does not confer menin-inhibitor sensitivity.

- blocker: NPM1 wild-type; the approval is restricted to susceptible NPM1 mutations
- blocker: KMT2A amplification is a copy-number gain and does not create the menin-KMT2A dependency the drug exploits

Biomarker gate. NPM1 mutation susceptible NPM1 mutation (threshold not met)

Ziftomenib is an approved menin inhibitor, but its label is confined to relapsed/refractory AML with a susceptible NPM1 mutation, and this disease is NPM1 wild-type. As with revumenib, the KMT2A amplification here is a copy-number gain rather than the fusion that would create menin-inhibitor sensitivity. The drug is approved but the target is absent, so it is assessed and set aside.

Key evidence.

- KOMET-001 (R/R NPM1-mutant AML) · phase 1/2, open-label single-arm · n=112 · CR+CRh 21.4%, median duration of CR+CRh 5.0 months

Toxicities that would change the decision. Differentiation syndrome, the menin-inhibitor class effect

Alongside the targeted options. Independent of the targeted options. The second approved menin inhibitor, listed alongside revumenib so the report is explicit that the menin-inhibitor class does not apply to an NPM1-wild-type, KMT2A-amplified genotype. It does not interact with the ranked options.

Last verified. 2026-07-24

References. fda:Komzifti; pmid:41653248; nct:NCT04067336; guideline:NCCN AML v3.2026

CPX-351 (liposomal daunorubicin/cytarabine)

Priority: low **Modality:** Systemic chemotherapy **Line:** Not line-indexed **Intent:** curative intent

What makes it standard.

- FDA On-label approval (different population · FDA approval 2017-08)
 - covers: Newly diagnosed therapy-related AML or AML with myelodysplasia-related changes in adults and pediatric patients 1 year and older.
- NCCN Category 2A (newly diagnosed AML-MR / t-AML) (different population · NCCN AML v3.2026)
 - covers: Newly diagnosed AML with myelodysplasia-related changes or therapy-related AML.

Fit to this patient. Likely ineligible The histology, AML-MR, matches the label, but the line does not: CPX-351 is approved for newly diagnosed AML-MR, and this patient is relapsed and refractory after transplant. Prior idarubicin in FLAG-IDA also adds cumulative anthracycline exposure.

- blocker: Approved for newly diagnosed AML-MR / t-AML only; the patient is relapsed/refractory
- blocker: Prior anthracycline (idarubicin) exposure adds cumulative cardiotoxicity

CPX-351 is the standard first-line intensive choice for the AML-MR histology this patient carries, which is why a reader would look for it. Its approval is confined to newly diagnosed disease, and this patient is relapsed and

refractory after a transplant, so the histology match does not open the drug here. It is assessed and set aside on line, not on target.

Last verified. 2026-07-24

References. fda:Vyxeos; guideline:NCCN AML v3.2026